

BOARD-CERTIFIED IMPLANT DENTISTRY

Dr. Jeries N. Qoborsi

DDS · DIPLOMATE, ABOI/ID

Bring the surgery your practice refers out back in-house: wisdom teeth, immediate implants, full-arch, and IV sedation, all on one scheduled day a month.

DIPLOMATE · ABOI/ID

FELLOW · AAID

IV SEDATION · CA

FULL-ARCH SURGERY

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SURGICAL IMPLANT DENTISTRY

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THE OPPORTUNITY

The production your practice refers away every month

Every wisdom-teeth referral, every implant consult sent across town, every denture patient who could have had a fixed arch – that revenue leaves your practice, and the patient experiences another office's care. Some never come back. And your patients never wanted to go: they trust *your* office. Every owner knows the question that follows a referral – **"Doctor, are you sure you can't do it here?"** Now the answer is yes.

\$2,400+

Typical production for one impacted third-molar case with IV sedation

\$5,000+

Surgical and restorative production per implant site kept in-house

\$25,000+

Full-arch patient value that walks out the door with every referral

THE PARTNERSHIP, IN ONE SENTENCE

I bring board-certified surgical care, IV sedation, and every piece of equipment into your operatories on a scheduled monthly surgery day. Your patients stay home, and your practice keeps the production.

Built for practices that:

- Refer out three or more surgical cases a month
- Own a CBCT that isn't producing surgical revenue
- Want to market implants and full-arch credibly
- Have anxious patients who need sedation to say yes
- Want to add surgery without putting anyone on payroll
- Belong to a group standardizing in-house surgery

THE SURGERY DAY

Turnkey. Your brand, start to finish.

A surgery day should feel like your practice gained a surgical department overnight, not like hosting a guest. The system below runs the same way at every partner office.

BEFORE

Your book holds a standing surgery date each month. To schedule a patient, your team emails me the pano, age, and a one-line health summary; I approve every case and assign codes in advance, so there are never surprises in the chair. Your coordinator receives fee guidance, consult checklists, and scripts so the day is built and pre-collected before I arrive.

DAY OF

I arrive with the surgical setup: instruments, motor, handpieces and burs, sedation and monitoring equipment, and a PRF centrifuge. One of your assistants joins me; my protocols handle the rest. Every case is documented in your practice software, under your roof.

AFTER

Every surgical patient receives my direct line for post-operative questions, so your front desk is never left holding a complication call. Follow-ups are scheduled into the next surgery day, and restorative hand-off notes go to your doctors.

WHAT I BRING

- Surgical instrumentation (sterilized sets)
- Implant motor, handpieces & burs
- IV sedation & monitoring equipment
- PRF centrifuge & venipuncture supplies
- Emergency protocols & medications
- My own malpractice coverage

WHAT YOUR OFFICE PROVIDES

- Patients currently being referred out
- An operatory and one assistant
- CBCT imaging & intraoral scans
- Scheduling & treatment coordination
- Pre-collection per your fee schedule
- Implants & consumables, ordered ahead and cost-shared

A complete surgical service line, inside your practice

Every case is screened in advance from your CBCT and intraoral scans. I operate; your doctors keep the restorative. And I am not here to replace your oral surgeon: I keep the predictable 90% of cases in your office and keep referring the unpredictable 10% to them. That case-selection discipline is why surgery days run safely and on schedule.

01 Wisdom Teeth & Surgical Extractions

Full and partial bony impactions removed with a minimally invasive, PRF-supported protocol for faster recovery. IV or oral sedation for every anxious patient.

The highest-volume referral leak in general practice, and the easiest to keep in-house.

02 Implant & Immediate-Implant Surgery

Immediate placement at the time of extraction, finished with socket-sealing abutments that preserve the gingival architecture and set patients up for the best possible final restoration. Fully guided workflows are available whenever the case benefits, with immediate provisionalization when primary stability allows.

Your doctors keep every implant crown, or I restore – whichever your practice prefers.

03 Full-Arch & All-on-X Reconstruction

Complete surgical phase for fixed full-arch cases – extractions, alveoloplasty, implant placement, and conversion support – planned digitally from the patient's CBCT.

Turn your denture patients into the most valuable cases in your practice.

04 Site Development & Grafting

Lateral-window and crestal sinus augmentation performed with piezosurgery, guided bone regeneration, ridge preservation, and PRF/sticky-bone protocols that set up predictable implant outcomes.

05 Oral & IV Sedation

Oral sedation protocols keep a full surgery day affordable and efficient for your patients, and state-permitted IV moderate sedation with full monitoring is available where your patients elect it. Sedation is its own growth engine: fearful patients accept treatment they have postponed for years.

Fewer than one in twenty general practices can offer this. Yours can.

CREDENTIALS

Credentials your patients and your marketing can lean on

When a partner office introduces me, they introduce a board-certified implant dentist. That certification belongs in your patient conversations, your website, and your case acceptance.

BOARD CERTIFICATION

Diplomate, American Board of Oral Implantology / Implant Dentistry

The highest credential in implant dentistry, awarded after written and oral examination of documented cases and held by only a few hundred dentists worldwide. Certified 2024 · Certificate No. 783 · Recognized by the American Board of Dental Specialties.

2025

Fellow, American Academy of Implant Dentistry

Awarded for documented implant experience, examination, and case presentation.

2023

Fellow, American Academy of Implant Prosthodontics

Recognition of exceptional skill in implant prosthodontics – surgery guided by the final restoration.

2022

IV Moderate Sedation Permit

Dental Board of California. Trained with full emergency-readiness protocols.

2021

DDS, Loma Linda University

Surgical and prosthodontic focus across a multi-office group from graduation onward.

Advanced surgical training

- AAID MaxiCourse – comprehensive implantology curriculum
- Advanced PRF education – Dr. Richard Miron
- All-on-X, guided surgery, GBR & sinus – Implant Ninja series
- Third molar surgery – Dr. Nekky Jamal
- Hands-on cadaver coursework – Implant Pathway, Dallas
- Ongoing full-arch & advanced grafting education

Primary-source verification of every credential is available on request. Malpractice coverage, NPI, W-9, and a complete credentialing packet are delivered within 48 hours of partnership.

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The economics of one surgery day

A straightforward model: 50/50 on surgical production with shared material costs, settled the day of service. No salary, no benefits, no payroll, and no surgical equipment to buy. I am paid only when your practice produces.

ILLUSTRATIVE SURGERY DAY	CASES	TYPICAL FEE	PRODUCTION
Impacted third molars with IV sedation	3	\$2,400	\$7,200
Immediate implant with grafting	2	\$2,800	\$5,600
Ridge preservation graft	1	\$600	\$600
One surgery day – new in-house production			\$13,400

The restorative stays yours

Implant crowns at \$2,500–\$3,500 per unit remain 100% your production, on implants placed in your own chairs.

The patient stays yours

Hygiene, recall, and the patient's family never experience another office. Referred patients don't always come back.

The math compounds

One anchor day a month at this pace keeps \$150,000+ of annual production in-house before restorative – offices running this model typically add six figures a year in new collections after my share.

Figures are illustrative, based on typical Southern California fee schedules; your office sets all fees and collects directly. Surgery days are confirmed at a minimum scheduled production level so the day is always worth your team's preparation.

GETTING STARTED

Three steps to your first surgery day

No long contracts, and no commitment before you have seen a surgery day run in your own office.

1

A fifteen-minute fit call

We review what you currently refer out, your fee schedule, and your equipment. You will know on this call whether the program fits, and what a surgery day would produce for your practice.

2

A no-commitment demonstration day

Your team books 3-4 pre-screened cases from charts you already have; I bring everything else and train your assistant that morning. You watch a surgery day run in your own office. If your team doesn't love it, we stop there – no obligation.

3

A guaranteed anchor day

Partner offices receive a recurring monthly surgery day, priority scheduling for urgent cases, and the systems that keep the pipeline full – including a recall-letter program that books surgery days from patients already in your charts.

AVAILABILITY

I keep a deliberately small roster of partner offices so every office is guaranteed its monthly day. Current openings are limited to select Mondays, Fridays, and Saturdays.

RESERVE A FIT CALL

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COVERAGE

Orange County · Los Angeles

Inland Empire · San Diego